INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab intravenous (IV) · weekly · metastatic

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00201
Version 6

IV trastuzumab every 7 days, continuing while beneficial and tolerated. This weekly NCCP card is for metastatic disease, without a fixed one-year course.

Treatment stages

01 **Current HER2 treatment**
Weekly visits

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report breathlessness, swelling, rapid weight gain, palpitations or persistent diarrhoea; tell staff immediately about a reaction during treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

The medicine in this card

Weekly trastuzumab targets HER2-positive metastatic breast cancer to help control it. Review symptoms, treatment benefit and cardiac function regularly.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Attend planned echoes. Report breathlessness, ankle swelling, rapid weight gain or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This weekly intravenous treatment targets HER2-positive breast cancer that has spread. Your team considers previous treatment, current cancer control, side effects and heart health when deciding whether to continue or change it.

Why this schedule?

The weekly and three-weekly schedules have different doses and visit patterns. Ask why the weekly schedule fits your plan and whether this card covers one part of a wider treatment combination. There is no automatic one-year stopping point for this metastatic pathway.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

Your treatment aim

Setting and duration

IV trastuzumab every 7 days, continuing while beneficial and tolerated. This weekly NCCP card is for metastatic disease, without a fixed one-year course.



Medicine identity

This card describes trastuzumab. Pertuzumab, taxanes, endocrine medicines and antibody–drug conjugates require their own treatment instructions.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Where this treatment fits

Trastuzumab IV · weekly · metastatic

7-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

WEEKLY VISITS

Current HER2 treatment

Repeated cycles

IV trastuzumab every 7 days, continuing while beneficial and tolerated. This weekly NCCP card is for metastatic disease, without a fixed one-year course.

Early and metastatic pathways have different aims and endpoints. The team confirms the applicable plan.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. NCCP remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

Check	Before first dose	Early visits	Week 6 · around C7	Week 12 · around C13	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment; earlier for concerns. Agree cardiac follow-up when treatment stops or changes.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Additional tests if needed	Team / companion-drug plan	Team / companion-drug plan	Repeat according to the team / companion-drug plan; no fixed universal interval in this NCCP card.

Check	Before first dose	Early visits	Week 6 · around C7	Week 12 · around C13	Later / after stopping
electrocardiogram (ECG) / rhythm	Baseline ECG in EMA cardiac assessment; it does not replace echo.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Record disease sites and scan purpose	Assess symptoms at every visit	Scan if due or clinically indicated	Response imaging under the individual plan	Metastatic response scans commonly every 2–4 months; markers are supplementary.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	If endocrine / ovarian suppression is planned: assess need	Not a routine trastuzumab or T-DM1 test alone	Timing depends on risk / result	Separate from the echo schedule	DEXA and repeat timing depend on endocrine treatment, fracture risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00201 v6 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · cardiac assessment and formulation

THE WHOLE TREATMENT JOURNEY

Trastuzumab IV · weekly · metastatic: the whole pathway

HER2-positive metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00201	Confirm setting, prior doses and formulation	Weekly IV trastuzumab	Review benefit, symptoms and heart function	Continue while beneficial and tolerated	Review cancer control and tolerance; agree continuation or a change.
Surgery METASTATIC	Local procedures only if indicated	No automatic curative surgical sequence	Coordinate procedures with oncology	Symptom / site-specific decisions	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Timing depends on the clinical need	Agree any hold / continuation with radiation oncology	radiotherapy (RT) is site / symptom directed. Agree any treatment hold with both teams.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00201	Baseline FBC, renal, liver	Repeat according to the team / companion regimen	According to the team / companion regimen	Repeat according to the team / companion regimen	Follow abnormalities and the next treatment plan
Heart checks NCCP 00201 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C13 for an uninterrupted weekly course	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail.
Scans / markers EARLY · METASTATIC · MARKERS	Define baseline sites and scan purpose	Review symptoms at each visit	Response imaging when due; exact timing agreed for the known disease sites.	Continue the individual metastatic response-assessment plan.	Response scans commonly every 2–4 months; individualise. Markers supplement clinical review and imaging.

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Endocrine / bone health EARLY · BONE	Check hormone receptors, previous treatments and bone-health needs	Any endocrine treatment requires a separate oncology plan	DEXA only if indicated by endocrine treatment or fracture risk	Do not add another anticancer medicine from this component card	Continue the individual bone and supportive-care plan

What each check is for

1

Blood tests

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here)

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

2

Echo / MUGA

Echo / MUGA every 12 weeks; around C13 for an uninterrupted weekly course

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

3

Cancer imaging and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used according to the treatment plan. A port insertion is a separate procedure.

STEP 3

Treatment and observation

First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit's approved policy.

STEP 4

After the visit

Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose. This medicine does not routinely require a take-home pump.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Before treatment and between visits

The team should arrange and review the tests.

Blood checks

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here)

Cardiac checks

Echo / MUGA every 12 weeks; around C13 for an uninterrupted weekly course. Baseline cardiac assessment includes ECG in EMA trastuzumab information. Further ECGs follow symptoms / cardiac plan; an ECG does not replace an echo.

After stopping

EMA trastuzumab information: cardiac assessment every 6 months after stopping, until 24 months; after anthracyclines in early disease, yearly to 5 years or longer if function falls.

The wider pathway

Coordinate cancer medicines with local treatment and follow-up.

Scans and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Radiotherapy relationship

Radiotherapy is used for a defined site or symptom when indicated. The oncology and radiation teams agree whether this medicine continues or pauses; do not infer a concurrent schedule from this card.

Bone health

If an aromatase inhibitor or ovarian suppression is planned, assess fracture risk and DEXA need. Repeat timing depends on results and bone treatment; this is not an automatic HER2-therapy scan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment	 **Day 1** Planned dose
Administration reaction	 **Day 1** During / after visit; report promptly
Fatigue / bowel changes	 **Days 1–7** Timing and severity vary
Heart / breathing symptoms	 **Days 1–7** Report on any day

No date entry needed: This is an orientation map; it does not calculate a personal appointment calendar.

Companion medicines: If another cancer medicine is prescribed, its side effects, tests and supportive medicines need its own plan.

Delayed treatment: Blood tests and echoes follow clinical due windows. A postponed dose does not automatically postpone monitoring.

> **Why these bands stay broad:** Treatment days show the planned schedule. Symptom bands are broad possibilities, not measured risk curves or guaranteed safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00201 v6 · official regimen ↗

> **There are no guaranteed "safe days".** New breathlessness, chest pain, collapse, severe allergic symptoms or sudden serious illness need urgent assessment on any day.

Between-treatment notes

Day 1–7: Your reference cycle

IV trastuzumab every 7 days, continuing while beneficial and tolerated. This weekly NCCP card is for metastatic disease, without a fixed one-year course. The team confirms each dose after the relevant checks. Contact them about a missed visit or new symptoms.

Support around the medicines

1 Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

2 Blood and heart appointments

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here). Echo / MUGA every 12 weeks; around C13 for an uninterrupted weekly course.

3 Other treatments

Any companion medicine, local procedure or radiotherapy needs its own oncology plan. This weekly metastatic component card does not prescribe an early-breast surgical sequence.

4 Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Questions worth asking

When is my next echo?

Echo / MUGA every 12 weeks; around C13 for an uninterrupted weekly course. Existing HER2 exposure and delays affect cycle numbering; the team should give the actual due window.

Does a tumour marker replace a scan?

No. In metastatic disease it may add information to symptoms and imaging. It should not alone determine treatment changes. In symptom-free early disease it is not routine recurrence screening.

Why was this treatment chosen for me?

This weekly intravenous treatment targets HER2-positive breast cancer that has spread. Your team considers previous treatment, current cancer control, side effects and heart health when deciding whether to continue or change it. The weekly and three-weekly schedules have different doses and visit patterns. Ask why the weekly schedule fits your plan and whether this card covers one part of a wider treatment combination. There is no automatic one-year stopping point for this metastatic pathway.

Try explaining the plan back: What is the purpose of your treatment? Which heart or blood test is due next, and which team arranges and reviews it?

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Trastuzumab IV · weekly · metastatic

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.
- First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit's approved policy.
- Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Trastuzumab intravenous (IV) · weekly · metastatic

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Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

Trastuzumab

New cough or breathlessness: seek prompt assessment.

2 Heart

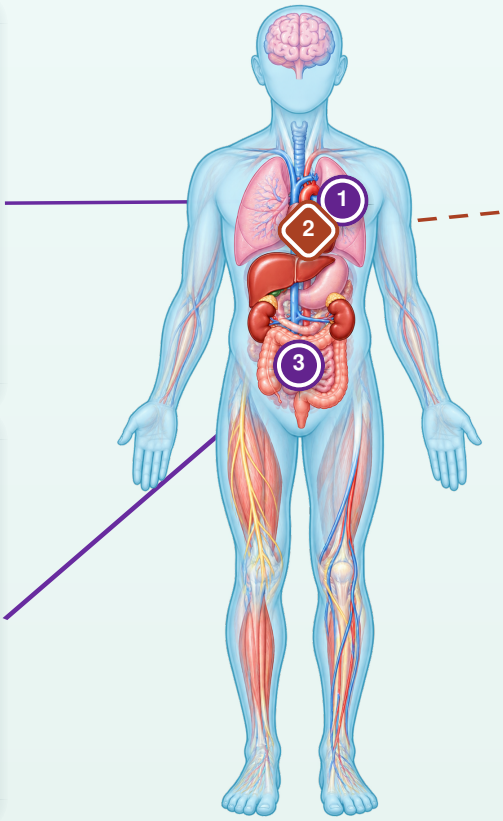
Trastuzumab

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.

3 Bowel / fatigue

Trastuzumab

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00201 v6 · official regimen ↗**
   https://healthservice.hse.ie/documents/6540/201_v6_Trastuzumab_IV_7_days.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
7. **EMA Herceptin product information · cardiac assessment and formulation**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk